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Fields of Concentration:
Health Economics, Industrial Organization

Qualifying Examinations Completed:
2023 (Oral): Industrial Organization, Econometrics

Dissertation Title: *Essays on Health Care Markets*

Committee:
Professor Jason Abaluck (Chair)
Professor Steven Berry
Professor Victoria Marone

Education:
Ph.D., Economics, Yale University, 2027 (expected)
M.Phil., Economics, Yale University, 2024
M.A., Economics, Yale University, 2023
B.A., Economics (with honors), University of Chicago, 2019
B.S., Mathematics, University of Chicago, 2019

Fellowships, Honors and Awards:
University and Teaching Fellowships, Yale University, 2021–2027
Department of Economics/Cowles Foundation/EGC Fellowship, Yale University, 2021–2026
Certificate of College Teaching Preparation (CIRTL Associate), Yale University, 2025
David A. Gimbel Fellowship, Yale University, 2025
Nathan Hale Associates Fellowship, Yale University, 2024
David S. Hu Award in Economics, University of Chicago, 2019
BFI Award for Academic Achievement in Macroeconomics, University of Chicago, 2019
Phi Beta Kappa, University of Chicago, 2019
President's Scholarship, University of Chicago, 2015–2019
Dean's List, University of Chicago, 2016, 2017, 2018

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Teaching Experience:

Fall 2025, Teaching Assistant to Prof. Benjamin Polak, Game Theory, Yale University
Spring 2025, Teaching Assistant to Prof. Daniel Hauser, Intermediate Microeconomics, Yale University
Spring 2024 and Fall 2024, Teaching Assistant to Prof. Cormac O'Dea, Introductory Microeconomics, Yale University
Fall 2023, Teaching Assistant to Prof. Evangelia Chalioti, Intermediate Microeconomics, Yale University

Research Experience:

Research Assistant to Prof. Zack Cooper, Yale University, 2022–2023
Research Assistant to Profs. Tarek Hassan and Leonardo Bursztyn, University of Chicago, 2017–2019

Work Experience:

Economist, Lyft, 2019–2021
Economist Intern, Lyft, 2018

Publications:

"High-Frequency Location Data Show That Race Affects Citations and Fines for Speeding" (2025) with Pradhi Aggarwal, Alec Brandon, Ariel Goldszmidt, Justin Holz, John List, Ian Muir, and Gregory Sun. *Science*, 387(6741), 1397–1401.

Working Papers:

"The Value of a Hospital Obstetric Unit: Access, Costs, and Health," *Job Market Paper*
"Redesigning Graduate Medical Education Payments: The Role of Residency Caps"
"Incidence of Drug Price Regulations" with Genna Liu

Seminar and Conference Presentations:

2026: Allied Social Science Associations Annual Meeting, Econometric Society North American Summer Meeting, Geography and Health: Exploring Rural-Urban Disparities (NBER Conference), American Society of Health Economists Annual Conference, Southeastern Health Economics Study Group

Referee Service:

American Economic Review

Professional Service:

Organizer, Industrial Organization Tea, Yale University

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References:

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Dissertation Abstract

The Value of a Hospital Obstetric Unit: Access, Costs, and Health [Job Market Paper]

Rates of OB provision, hospitals' decisions to operate obstetric (OB) units, are lower among rural hospitals than urban hospitals and declining overall. I study how OB provision affects access to care, the costs of delivering care, and maternal and infant health in equilibrium. Then, I quantify the net externalities of OB provision and evaluate policies aimed at preventing OB unit closures.

In a simple example, I show that an OB unit increases welfare by giving mothers access to an additional option, but it also decreases welfare due to its fixed cost of operation. Effects on variable costs and maternal and infant health are ambiguous, depending on the OB unit's marginal cost and quality relative to its competitors. The OB hospital does not fully internalize its effects on mothers, infants, or other hospitals, so OB provision has a net externality that can be positive or negative.

I study these effects in Arkansas, a highly rural state. Using data on births and medical claims from the Arkansas All-Payer Claims Database, I estimate a model of the hospital OB care market. First, hospital systems make OB provision decisions. Second, insurers and hospital systems bargain over prices. Third, mothers choose OB hospitals, and maternal and infant health is determined.

I apply the estimated model to simulate equilibrium outcomes if each OB unit were to close. The access-to-care effect of OB provision is large, while effects on health and variable costs are small and mixed in sign. The average OB unit generates a positive externality worth $104 per Arkansas birth, mostly driven by access to care. Mothers highly value having more options and shorter travel distances, but OB hospitals do not internalize much of this value.

Finally, I evaluate policies, starting with Arkansas's 2025 Healthy Moms, Healthy Babies Act, which raises the prices paid by Medicaid for OB care by 70%. Under realistic fixed cost increases, the policy keeps OB units open and improves welfare, primarily through access to care. Its marginal value of public funds ranges from 1.0 to 1.5: each dollar of public spending returns a dollar plus $0 to $0.50 in additional welfare. However, a drawback of the Medicaid price hike is that it pays more to large hospitals that would have operated OB units anyway. As an alternative, I consider a flat subsidy that pays all OB hospitals the same amount. The subsidy's marginal value of public funds ranges from 1.2 to 4.2, achieving the same goal more cost-effectively.

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Redesigning Graduate Medical Education Payments: The Role of Residency Caps

Physicians in the U.S. must complete graduate medical education (GME) by training as residents in teaching hospitals. Teaching hospitals receive GME payments from Medicare, with each hospital assigned a cap on its subsidized residents. About half of teaching hospitals exceed their caps and thus receive no additional payments for training more residents. I study whether GME payments can be redesigned to increase the number of residents by redirecting payments from over-cap to under-cap hospitals. Instrumenting for under-cap status using changes in hospital size since the caps were assigned, I find that under-cap hospitals' resident counts respond to payments with an elasticity of 0.74, while over-cap hospitals have no response. My estimates suggest that reducing over-cap hospitals' payments by 25% and transferring the savings to under-cap hospitals would lead to 3,700 more residents without any change in government spending. These gains are proportionally larger in rural areas, which face more severe physician shortages.

Incidence of Drug Price Regulations, with Genna Liu

Prescription drug prices are determined through a chain of interactions among manufacturers, insurers, and consumers. A regulator seeking to lower drug prices must choose which point in the chain to regulate and which drugs to target. How are the effects of drug price regulations distributed among market participants, and how does that guide a regulator's preferred choices? We develop a model of a drug market with rebate bargaining and community-rated premiums. On the one hand, a net price ceiling transfers surplus from manufacturers to insurers. On the other hand, a copay cap reaches drug users directly, and insurers and manufacturers split its cost according to bargaining power. Beyond that, consumers are affected only through premiums, which only move when drug users are marginal enrollees. A consumer-welfare-maximizing regulator should cap copays, while a spending-minimizing regulator can use either instrument. Copay caps should target drugs with high out-of-pocket prices, many users, and elastic utilization. Net price ceilings should target drugs with high net spending. Applied to insulin in Medicare Part D, the model implies that Medicare's $35 insulin copay cap raised fills by 3%, at a cost borne by insurers and manufacturers rather than by other consumers.